

Anaemia Investigation Pathway

Source: Neuron Clinical Protocol Library

Initial assessment

Confirm a low haemoglobin result with a repeat full blood count before starting any investigation. Record symptom duration, fatigue severity, dizziness, and any reported bleeding. Review medicines that can contribute to anaemia.

Iron studies

Request ferritin, serum iron, transferrin saturation, and B12 with folate when haemoglobin is below the reference range. A ferritin below 30 micrograms per litre indicates iron deficiency and requires a source of blood loss to be identified.

Escalation

Haemoglobin below 80 grams per litre, or any haemoglobin fall with chest pain, breathlessness at rest, or syncope, requires same-day clinical assessment. Arrange urgent review rather than routine follow-up.

Follow-up

Recheck the full blood count six to eight weeks after starting oral iron. Refer to gastroenterology when iron deficiency anaemia is confirmed in adults without an obvious cause.